

Pharmacists Council of Nigeria (PCN)
PCN Standards for Monitoring Registered Premises

A Paper by

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Introduction

The repositioning plan by the Registry for more effective regulatory control of the profession by the Pharmacists Council of Nigeria (PCN), submitted to and approved by the Governing Council in 2004, gave birth to the Inspection and Monitoring Department.

Previously, the Department was responsible only for the inspection of premises already registered for one form of pharmaceutical activity or another, as well as for new premises seeking approval. Inspection, as we all know, focuses on personnel and equipment in the environment (the premises).

However, the mere presence of these elements in any premises does not automatically translate into excellent service to the public. In other words, well-kept premises with the required facilities and well-trained personnel may not necessarily guarantee high-quality professional service.

Inspection vs. Monitoring

Monitoring, on the other hand, focuses on the actual delivery of professional services to patients within the premises. It evaluates how core services are rendered by utilizing the instruments, facilities, and personnel earlier observed during the inspection phase.

A premises under monitoring must have previously been inspected, approved, and licensed by the Council. Thus, the distinction is clear: **inspection** is about compliance with physical and structural requirements, while **monitoring** assesses the quality of pharmaceutical care delivered.

A Compendium of Minimum Standards of Practice

The search for benchmark standards in pharmacy practice led the Registry to initiate the compilation of **Minimum Standards of Practice for the Assurance of Pharmaceutical Care** across the following areas:

- Hospital Pharmacy
- Retail Pharmacy
- Wholesale Pharmacy
- Importation and Manufacturing

This initiative was approved by the Governing Council, resulting in a four-part compendium that defines the practice in a standardized manner. The formal presentation of the compendium was scheduled for August 18, 2005, during the commissioning of the PCN Head Office/Registry by the Honourable Minister of Health.

The compendium ensures that every inspected, approved, and licensed premises has access to minimum standards. Pharmacists can use it to gauge expectations, while inspectors can rely on it as a tool to evaluate compliance and service quality.

Monitoring Activities

Monitoring activities are carried out through the following methods:

- **Observation** of interactions between the pharmacist and patients.
- **Discussions** between the inspector and the pharmacist on practice-related issues to evaluate competence and adherence to professional standards.
- **Reference to previous inspection reports** to assess continuity of compliance.

The goal is to ensure that practices meet or exceed benchmark standards. Falling below such standards is considered a failure in professional service delivery.

The Future of Monitoring

- Monitoring is expected to provide a **two-way feedback system** between licensed premises and the regulatory body on the adequacy of services rendered.
- It is the responsibility of PCN to ensure society receives the pharmaceutical care it deserves. This is one reason why the **Mandatory Continuing Professional Development (MCPD)** programme was introduced to update pharmacists' knowledge.
- With monitoring fully established, PCN will design **specialized continuing development programmes** tailored to different practice areas (hospital, retail, wholesale, importation, and manufacturing).
- Updated versions of the compendium will serve as reference materials for continuing professional development.

- PCN also has the responsibility to periodically update the knowledge base of licensed premises, outside MCPD cycles, to ensure that patients consistently receive quality services.

The Nigerian public is becoming increasingly sophisticated, demanding only the best. Patients may seek second opinions or even resort to legal action if professional service falls short of expectations. To protect both patients and pharmacists, monitoring must be meaningful, comprehensive, and proactive.

The best time to entrench this system is now.

Thank you all, and God bless.

Mandatory Continuing Professional Development (MCPD): Successes and Challenges

A Lecture by

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At the One-Day MCPD Train-the-Trainer Workshop

Fred Adenika Auditorium Hall, PCN, Idu, Abuja

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1.0 Introduction

1.1 The World Health Assembly Resolution WHA 27.31 recognized Continuing Education as an integral part of the health and education system, highlighting its importance to health authorities in achieving and maintaining high-quality healthcare.

1.2 The need for **Mandatory Continuing Professional Education** in the recertification of health professionals was acknowledged at the maiden National Health Summit in 1995. Subsequently, in 1996, the Honourable Minister of Health directed all professional regulatory bodies to begin discussions on modalities for the programme's early implementation.

1.3 In line with government directives, the Pharmacists Council of Nigeria (PCN) commenced the first cycle of **Mandatory Continuing Professional Development (MCPD)**—formerly known as MCPE—for the recertification of pharmacists in April 1998, with a planned completion date of December 31, 2013. PCN was the first healthcare regulatory body in Nigeria to initiate MCPD. Defaulting pharmacists were given a 16-month grace period (January 2004 – April 2005) to complete the three required modules.

1.4 The Federal Ministry of Health has since accepted the need for mandatory recertification of all health professionals. Processes are ongoing to define policy, regulations, and legal frameworks to make MCPD a statutory requirement for recertification.

2.0 Objectives of MCPD

The objectives of MCPD are to:

- a. Update pharmacists' knowledge and keep them abreast of advances in pharmaceutical science and pharmacy practice.
- b. Supplement initial training, expanding knowledge and skills to meet consumer needs.
- c. Improve skills to ensure pharmacists remain relevant members of the healthcare team.
- d. Provide a platform for sharing ideas and experiences, enhancing competence and commitment, and improving the quality of products and services.
- e. Serve as an ongoing process of adaptation, enabling pharmacists to contribute effectively to change in healthcare delivery.
- f. Ensure pharmacists remain statutorily fit to offer licensed services, in line with Section 14(1) and (6) of the Pharmacists Council of Nigeria Act 91 of 1992.

3.0 Overview of the First Cycle

3.1 From April 1998 to April 2005, a total of **6,870 pharmacists** participated in the MCPD programme, translating to **17,099 completed modules** (MCPD database, July 26, 2005).

3.2 Participation statistics as of July 26, 2005, showed varying levels of completion across the required modules.

3.4 Registry Records and Complaints

A set of three lists of pharmacists who had completed **three, two, and one MCPD modules** respectively was produced by the Registry and circulated in September 2004 to:

- All Directors of Pharmaceutical Services (DPSs),
- All MCPD Centres,
- The Federal Ministry of Health, Abuja, and
- The Pharmaceutical Society of Nigeria (PSN) National Secretariat, Lagos.

This circulation was intended to cross-check records available in the Registry and allow pharmacists to lodge complaints where necessary. Many complaints were received (and are still being received), and their treatment is ongoing at the Registry.

As a result of this process, an updated list of participants who had completed the three MCPD modules was produced and circulated to all DPSs in March 2005.

4.0 The Present Focus of the Programme

4.1 Change of Nomenclature

The Council, in its repositioning of the Registry, decided to change the nomenclature from **Mandatory Continuing Professional Education (MCPE)** to **Mandatory Continuing Professional Development (MCPD)**.

This change was based on the understanding that pharmacists have already been educated; what is now required is continuous **development**—updating their knowledge and skills to keep pace with advancements in pharmaceutical science and modern trends in pharmacy practice.

4.2 MCPD Curriculum and Credit Units

The MCPD curriculum covers core and elective areas with assigned credit units as follows:

- **Public Health Pharmacy** – 6 Credit Units
- **Health Education and the Pharmacist** – 2 Credit Units
- **Information Technology and Pharmacy** – 2 Credit Units
- **Contemporary Issues** – 4 Credit Units
- **Pharmacy Practice** – 4 Credit Units
- **Pharmacy Laws and Consumer Rights** – 5 Credit Units
- **Recent Advances in Management and Therapy of Prevalent Diseases** – 3 Credit Units
- **Principles of Management** – 2 Credit Units
- **Pharmaceutical Sales and Marketing** – 2 Credit Units
- **Laboratory Investigations** – 2 Credit Units
- **Total Quality Assurance** – 3 Credit Units

Special Electives (existing CEP activities of PSN and related bodies) – 14 Credit Units

4.3 Feedback and New Direction

A general appraisal of the first MCPD cycle highlighted the need to make programmes more **practical, participatory, and interactive**.

This led the Governing Council to organize a **Train-the-Trainer (TOT) Workshop** to:

- Prepare resource persons,
- Standardize delivery across MCPD centres, and
- Strengthen monitoring and feedback mechanisms.

4.4 Involvement of Practitioners

The Council recognized the importance of engaging **experienced practitioners** from diverse fields—not just academics—as resource persons. Their expertise would complement the scholarly contributions of universities and broaden the programme's relevance to real-world practice.

4.5 Scope of the TOT Workshop

Participants in the workshop were drawn from:

- All 11 MCPD Centres,
- The Pharmaceutical Society of Nigeria (PSN),
- Technical groups outside academia,
- Zonal offices, and
- The PCN Headquarters/Registry, Abuja.

4.6 Objectives of the TOT Workshop

The objectives of the Train-the-Trainer Workshop included:

- Improving the quality of seminar delivery and overall programme organization.
- Ensuring acceptable uniformity of MCPD programmes across centres.
- Providing feedback to MCPD centres for self-evaluation and continuous improvement.
- Strengthening coordination between PCN and MCPD centres.

- Making recommendations for a sustainable MCPD programme for registered pharmacists in Nigeria.
- Enhancing the capacity of participants to cascade knowledge to other resource persons.

5.0 Further Challenges

Being the pioneer regulatory body in Nigeria to implement training linked to professional recertification, PCN must continuously refine MCPD—particularly in **programme delivery**.

The psychology of **adult learning** must be factored into MCPD design. If teaching remains mechanical, even well-structured topics may fail to achieve their objectives. Adult learners require interactive, participatory approaches to derive value and maintain engagement.

The **second MCPD cycle** aims to correct lapses experienced in the first by introducing this new focus. Presentations should evolve into **interactive exercises** that foster understanding, engagement, and professional satisfaction for participants.

Thank you all, and God bless.